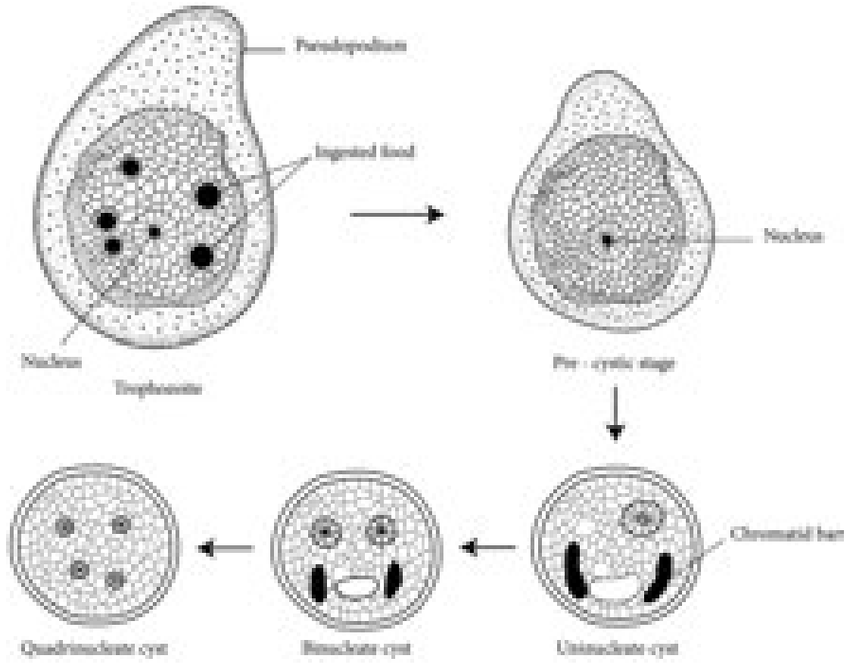


Figure 8.1: Trophozoite, precyst and cyst of *Entamoeba histolytica*

- The cyst that are swallowed along with food and water enters into the alimentary canal. The cyst wall is resistant to action of gastric juice. The cyst pass through the stomach undamaged and enters the small intestine (Figure 8.2).
- When the cyst reaches caecum or lower part of the ileum, due to alkaline medium, the cyst wall is damaged by trypsin leading to excystation.
- The cytoplasm gets detached from the cyst wall and an amoeboid movement appear causing a tear in the cyst wall, through which quadrinucleate amoeba is liberated. This stage is called the metacyst.
- The nuclei in the metacyst immediately undergo division to form 8 nuclei, each of which gets surrounded by its own cytoplasm to become 8 small amoebulae or metacystic trophozoites.
- These metacystic trophozoites are carried to the caecum and colon. They invade the tissues and lodge in the submucous tissue of the large intestine which is their normal habitat.
- Trophozoite grow and multiply by binary fission. The trophozoite phase of the parasite is responsible for producing the characteristic lesion of amoebiasis.



The amoeba infecting man may be classified according to their pathogenicity and habitat.

A. Pathogenic

Intestinal Amoeba: *Entamoeba histolytica*

B. Non pathogenic

1. Mouth Amoeba: *Entamoeba gingivitis*
2. Intestinal Amoeba: *Entamoeba coli*
   Entamoeba nana

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